NCCP 00806 · VERSION 2

# Your treatment, explained

**NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma.** · NCCP 00806 v2 · Document revised 5 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Cisplatin, lomustine and vincristine (CLV)

42-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call promptly for fever, bleeding, new cough or breathlessness, persistent vomiting or reduced urine. Report hearing, nerve or bowel changes before the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · lomustine-smpc · cisplatin-smpc · nci-cns

YOUR COMPLETE REFERENCE

## Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your whole treatment course
Your 42-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your treatment and its purpose
Taking treatment and organising visits
Safety tests, support and urgent symptoms
Understanding your particular brain tumour
Tests, scans and recovery after treatment
Brain symptoms, medicines and when to call
Your overall treatment plan and choices
Living well, rehabilitation and support
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

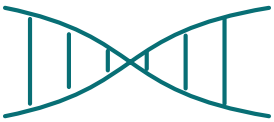
# How your medicines work

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

ORAL CHEMOTHERAPY CAPSULE

## Lomustine

CCNU



### Damages DNA

Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

**What to watch for**

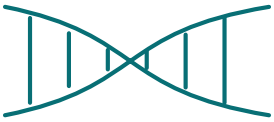
This is one dose on Day 1, not a daily medicine. If an extra dose is taken, get urgent advice even if you feel well. Keep blood tests after the dose and report fever, bruising, bleeding, new cough or breathlessness.

Sources: nccp · lomustine-smpc

CHEMOTHERAPY INFUSION INTO A VEIN

## Cisplatin

Platinum chemotherapy



### Forms DNA links that disrupt cell division

Cisplatin forms chemical links within DNA, making it harder for cells to copy and repair their genetic material. It can damage tumour cells but also affect kidney function, hearing, nerves and blood-forming cells. Fluids, electrolyte checks and anti-sickness medicines are part of the treatment plan to reduce avoidable harm.

Platinum enters cells → DNA links form → Cell growth disrupted

### What to watch for

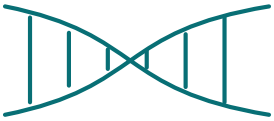
Report ringing or reduced hearing, numbness or weakness, reduced urine, persistent vomiting or difficulty drinking. Tell the team about fluid restrictions or heart and kidney conditions before hydration.

Sources: nccp · cisplatin-smpc

CHEMOTHERAPY GIVEN INTO A VEIN

## Vincristine

Vincristine infusion



### Disrupts the machinery used for cell division

Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Infusion into a vein → Cell division disrupted → Nerves and bowel may be affected

### What to watch for

Report tingling, weakness, walking difficulty or constipation before the next dose. Tell staff immediately if the infusion site stings, burns or swells.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

# Your anatomical side-effect map

## A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

### 1 Brain / hearing

**Cisplatin / tumour**

Report new ringing or reduced hearing promptly. New seizure, sudden weakness or speech difficulty needs emergency help.

### 3 Blood / infection

**Chemotherapy**

Counts can fall weeks after lomustine. Fever, sudden illness, unusual bruising or bleeding needs urgent advice.

### 5 Kidneys / hydration

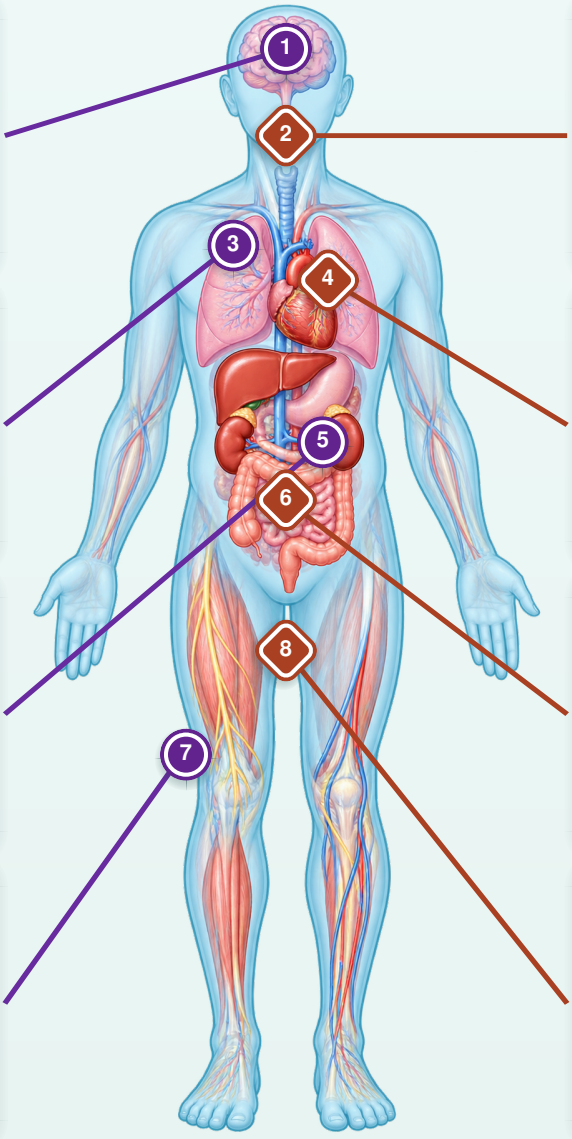
**Cisplatin / lomustine**

Attend kidney and electrolyte checks. Follow your individual fluid plan; report reduced urine, swelling or difficulty drinking.

### 7 Hands / feet / nerves

**Vincristine / cisplatin**

Report tingling, numbness, weakness, balance or hand-function changes before the next dose; do not wait for them to worsen.




### 2 Mouth / eating

**Chemotherapy**

Soreness or ulcers can limit intake. Use gentle mouth care and ask for help before eating or drinking becomes difficult.

### 4 Lungs

**Lomustine**

Report new cough or breathlessness. Keep the lung checks arranged; severe breathing difficulty needs emergency help.

### 6 Nausea / bowels

**Cisplatin / vincristine**

Use prescribed anti-sickness and bowel medicines. Severe pain, vomiting or no stool or wind needs urgent advice.

### 8 Fertility / pregnancy

**Chemotherapy**

Discuss preservation and contraception before starting. Do not breastfeed during treatment; ask for written precautions.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

## Your whole treatment course

BEFORE CLV

### Recovery after craniospinal radiotherapy

**Start window: 4–6 weeks after RT** **Safety and recovery review**

Record the actual RT finish date and specialist-approved chemotherapy start. Earlier surgery and RT have separate plans.

42-DAY CYCLE

### Cisplatin, lomustine and vincristine (CLV)

**Up to 8 cycles** **42 days: lomustine once + cisplatin IV D1; vincristine IV D1, 8 + 15.**

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

Each start, visit, delay or dose change is confirmed by your team. The card records treatment and does not authorise another dose.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

## Tests, symptoms and treatment decisions

Medicine-safety checks, tumour-tissue testing and response imaging answer different questions. Product-information additions are labelled. No universal blood tumour-marker panel or scan interval is implied.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00806 v2 · current Irish regimen · NCI: medulloblastoma diagnosis, risk groups and treatment · patient education · HPRA: cisplatin Accord current product information · July 2026 · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

## Cisplatin, lomustine and vincristine (CLV): treatment and checks

**NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma.**

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | During treatment | Between visits | Next treatment / response | After a pause or stop |
|---|---|---|---|---|---|
| **Bloods / medicine safety** NCCP 00806 | Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. | Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. | Keep additional tests even on days without an anticancer dose. | Team reviews current and previous results, symptoms and dose history. | Agree continuing checks; lomustine effects may persist. |
| **Neurology / medicines / function** NCCP 00806 | Record usual symptoms, medicines, daily function and support needs. | Review headache, seizures, speech, strength, cognition and any steroid plan. | Report new changes early; do not wait for an MRI. | Discuss treatment tolerance, priorities and the next authorised dose. | Agree rehabilitation, symptom support and contacts. |
| **Response / appointments** NCCP 00806 | Confirm exact diagnosis and baseline imaging. | Imaging and clinical review follow the specialist plan. | Record bookings, result-review contacts and questions. | Interpret scans with symptoms, treatment effects and medicines. | Agree the next imaging and clinical review. |

## What each check is for

1

### Recovery after craniospinal radiotherapy

Before CLV

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

**Ask / act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00806

2

**Cisplatin, lomustine and vincristine (CLV)**

42-day cycle

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

**Ask / act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00806

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.39

### Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

**Treatment timing: displayed days 1–42**

| Treatment or check | Days in this window | What happens |
|---|---|---|
| **Lomustine · one oral dose** | Day 1 | Take once on Day 1 only; use the newly authorised prescription. |
| **Cisplatin + prescribed hydration** | Day 1 | Day 1 with anti-sickness medicines, fluids and electrolyte plan. |
| **Vincristine infusion** | Day 1 | Report bowel and nerve symptoms before this visit. |
| **Vincristine infusion** | Day 8 | Report bowel and nerve symptoms before this visit. |

| Treatment or check | Days in this window | What happens |
|---|---|---|
| **Vincristine infusion** | Day 15 | Report bowel and nerve symptoms before this visit. |
| **Continuing blood-count checks** | Days 1–42 | Lomustine SmPC advises weekly counts for at least six weeks; arrange all checks. |
| **Symptoms and support between visits** | Days 1–42 | Keep the written emergency plan and do not wait for the next treatment to report changes. |

**A treatment record is not a prescription:** The team may delay, alter or stop a component after reviewing symptoms, blood tests, organ function and previous toxicity. Do not restart a held medicine or add a replacement dose yourself.

**Lomustine is one dose, not daily:** Only the authorised Day 1 dose belongs to this cycle. Do not take a further course before six weeks and a new team decision. Taking repeated daily doses can cause serious harm. If a dose is missed or vomited, contact the team or pharmacist before taking any more. If an extra dose is taken, seek urgent clinical advice straight away even if you feel well; do not wait for symptoms.

**Delayed marrow effects:** Current lomustine product information advises blood counts weekly for at least six weeks after a dose. Low counts may be delayed and cumulative, so keep checks even when you feel well or the next treatment is postponed.

**Why these bands stay broad:** Days without a scheduled dose still need symptom monitoring and planned tests. The calendar does not replace a treatment decision. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00806 v2 · current Irish regimen ↗

**There are no guaranteed "safe days".** Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

**Complete timing and symptom notes**

### Day 1: Three medicines and hydration

Lomustine once by mouth, cisplatin and vincristine by infusion. Day 1 includes the individual fluid and anti-sickness plan. Confirm hearing, kidney, bowel and nerve symptoms before treatment.

### Day 2–15: Vincristine visits on Days 8 and 15

There are no more scheduled lomustine or cisplatin doses in this cycle. Keep the bowel plan and report tingling or weakness. A held dose needs a new team decision; do not replace it yourself.

### Day 16–42: Recovery and continuing tests

Counts can fall after infusion visits finish. Keep weekly lomustine blood-count checks and the team's other appointments. Call for fever, bleeding, new respiratory symptoms or difficulty drinking.

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

### Taking the treatment

Take lomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Cisplatin and vincristine are given into a vein. Day 1 includes fluids before and after cisplatin; later visits on Days 8 and 15 are for vincristine. The total visit is longer than the infusion itself.

STEP 2

### Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

STEP 3

### Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

STEP 4

### Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

STEP 5

### Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

STEP 6

### Appointments and transport

Before starting: Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. During treatment: Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. Brain/spine imaging and any other response tests follow the individual specialist plan; no universal scan or tumour-marker timetable is set here.

STEP 7

### Help with the daily plan

Agree who can help with transport, prescriptions, phone calls and recording symptoms if memory, vision, weakness or fatigue makes them difficult. Your consent and preferences guide what is shared. Keep the printed schedule and actual urgent contacts together.

Sources: NCCP 00806 v2 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

## Support around the medicines

### 1 Anti-sickness plan checked for interactions

Cisplatin has high emetic potential; lomustine adds oral moderate-to-high risk. Use the team's prevention and rescue plan, including delayed nausea after Day 1. Aprepitant and some antifungals interact with vincristine, so the oncology pharmacist must select and check the plan; do not add medicines yourself.

### 2 Hydration and electrolytes

Cisplatin needs fluids before and after treatment and an individual electrolyte plan. Tell the team about fluid limits, swelling, heart or kidney conditions, vomiting and reduced urine. Follow your own home-drinking advice rather than a universal volume.

### 3 Prevent constipation and monitor nerves

NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.

### 4 Medicines, infection prevention and recovery

Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.

Sources: nccp · lomustine-smpc · cisplatin-smpc

TREATMENT REFERENCE · 01

# Your treatment and its purpose

Keep the exact diagnosis, regimen and treatment goal together.

## Why this treatment?

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

Sources: nccp

## What each medicine does

Lomustine: Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests. Cisplatin: Cisplatin forms chemical links within DNA, making it harder for cells to copy and repair their genetic material. It can damage tumour cells but also affect kidney function, hearing, nerves and blood-forming cells. Fluids, electrolyte checks and anti-sickness medicines are part of the treatment plan to reduce avoidable harm. Vincristine: Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Sources: nccp · lomustine-smpc · cisplatin-smpc

## Your schedule

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

Sources: nccp

## Evidence and uncertainty

The treatment strategy has a role in specialist multimodal care, but paediatric outcomes cannot be used as an adult survival estimate or applied to every embryonal tumour. The cited childhood study does not establish a numerical benefit for this exact adult high-risk population. Medulloblastoma and other embryonal tumours have different molecular and clinical contexts. Glioma trials and the uploaded glioma guide do not establish CLV efficacy. The neuro-oncology team must explain the evidence, uncertainties, treatment burden and alternatives for the individual diagnosis.

Sources: nccp · packer1994 · nci-medulloblastoma

### Consent and decisions

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. This schedule follows craniospinal RT and needs its own consent discussion.

Sources: nccp

TREATMENT REFERENCE · 02

## Taking treatment and organising visits

Different medicines and support plans have different days.

### Taking the treatment

Take lomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Cisplatin and vincristine are given into a vein. Day 1 includes fluids before and after cisplatin; later visits on Days 8 and 15 are for vincristine. The total visit is longer than the infusion itself.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

Sources: nccp · lomustine-smpc

### Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

Sources: nccp · lomustine-smpc

### Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Appointments and transport

Before starting: Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. During treatment: Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. Brain/spine imaging and any other response tests follow the individual specialist plan; no universal scan or tumour-marker timetable is set here.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Help with the daily plan

Agree who can help with transport, prescriptions, phone calls and recording symptoms if memory, vision, weakness or fatigue makes them difficult. Your consent and preferences guide what is shared. Keep the printed schedule and actual urgent contacts together.

Sources: nci-cns

TREATMENT REFERENCE · 03

# Safety tests, support and urgent symptoms

A treatment-free day is still part of the care plan.

### Which tests and when?

Before starting: Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. During treatment: Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. Brain/spine imaging and any other response tests follow the individual specialist plan; no universal scan or tumour-marker timetable is set here.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Anti-sickness plan checked for interactions

Cisplatin has high emetic potential; lomustine adds oral moderate-to-high risk. Use the team's prevention and rescue plan, including delayed nausea after Day 1. Aprepitant and some antifungals interact with vincristine, so the oncology pharmacist must select and check the plan; do not add medicines yourself.

Sources: nccp

### Hydration and electrolytes

Cisplatin needs fluids before and after treatment and an individual electrolyte plan. Tell the team about fluid limits, swelling, heart or kidney conditions, vomiting and reduced urine. Follow your own home-drinking advice rather than a universal volume.

Sources: nccp

### Prevent constipation and monitor nerves

NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.

Sources: nccp

### Medicines, infection prevention and recovery

Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.

Sources: nccp

### When to call

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call promptly for fever, bleeding, new cough or breathlessness, persistent vomiting or reduced urine. Report hearing, nerve or bowel changes before the next infusion.

Sources: nccp · lomustine-smpc · cisplatin-smpc · nci-cns

TREATMENT REFERENCE · 04

## Understanding your particular brain tumour

CLV here is for selected adult high-risk medulloblastoma or another eligible embryonal tumour; it is not a glioma regimen.

### The exact diagnosis matters

Medulloblastoma is an embryonal tumour that usually starts in the cerebellum. The NCCP regimen also names other primitive neuro-ectodermal tumours, including pinealoblastoma. Ask the specialist to record the current pathology name and explain any older wording. These diagnoses are not interchangeable with glioblastoma or other gliomas.

Sources: nccp · nci-medulloblastoma

### Risk group and spread

The team considers the exact tumour, what remains after surgery and whether it has spread within the brain, spine or cerebrospinal fluid. Ask what makes your tumour high risk and which results establish that. A general glioma grade explanation does not replace medulloblastoma risk assessment.

Sources: nccp · nci-medulloblastoma

### Molecular results

Medulloblastoma can be classified by molecular findings, including WNT, SHH and non-WNT/non-SHH groups. Ask which tests apply to your exact tumour and how they affect discussion. Molecular tests used for another tumour type are not automatically relevant to this diagnosis. If a result is pending or not applicable, record that clearly.

Sources: nci-medulloblastoma

### How the treatments fit together

Surgery obtains tissue and removes tumour where safely possible. Craniospinal radiotherapy treats planned areas of the brain and spine. This NCCP CLV schedule starts four to six weeks after craniospinal RT, after recovery and safety review. The team confirms the role of each part for your diagnosis.

Sources: nccp · nci-medulloblastoma

### A useful diagnosis record

Keep the pathology date and wording, risk group and its reasons, molecular results if relevant, surgery and RT dates, brain/spine scan findings and any cerebrospinal-fluid assessment. Ask who will explain pending results. Use this record to understand decisions, not to determine your own chemotherapy eligibility.

Sources: nccp · nci-medulloblastoma

TREATMENT REFERENCE · 05

## Tests, scans and recovery after treatment

Safety tests and tumour-response tests have different purposes.

### Bloods, kidneys and hydration

Blood counts check marrow recovery; kidney and liver tests help assess whether treatment is safe. Cisplatin also needs an individual hydration and electrolyte plan. Tell staff about fluid restrictions, swelling, heart or kidney problems and difficulty drinking. Do not use a generic home fluid target in place of your plan. Current cisplatin product information advises weekly blood-count, kidney/liver and electrolyte checks through treatment.

Sources: nccp · cisplatin-smpc

### Hearing, nerves and lung assessment

Cisplatin can affect hearing and nerves; vincristine can affect nerves and bowel function; lomustine can affect the lungs. Tell the team about changes before every visit. NCCP lists hearing and lung tests according to clinical need, while current lomustine product information advises baseline and continuing lung assessment. Ask which checks are booked for you. Current cisplatin product information advises a hearing test before treatment and repeat testing if hearing symptoms develop.

Sources: nccp · lomustine-smpc · cisplatin-smpc

### Brain and spine review

Medulloblastoma may involve the brain, spine or cerebrospinal fluid. Your specialist decides which imaging or other tests are needed at each stage. Keep the next scan date and result-review appointment in your passport. This guide does not set a universal scan, lumbar-puncture or blood tumour-marker timetable.

Sources: nccp · nci-medulloblastoma

### Delayed effects and rehabilitation

Symptoms may change after surgery, RT or chemotherapy. Hearing, balance, strength, memory, fatigue and daily function deserve follow-up as well as scans. Ask which longer-term checks and rehabilitation are relevant after your particular treatment. Report new changes promptly rather than waiting for a routine appointment.

Sources: nccp · nci-cns

### Keep results with decisions

Record when a test is booked, completed and explained, and the next agreed step. A normal result from one test does not replace another medicine-safety check. The team authorises treatment after considering symptoms, current results and earlier side effects.

Sources: nccp

TREATMENT REFERENCE · 06

## Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

### Know your usual symptoms

Write down your usual headache, strength, walking, speech, vision, memory, mood, sleep and seizure pattern before treatment. Note new changes, when they started and how they affect daily tasks. With your permission, a trusted person can help spot changes or record what happened.

Sources: nccn-brain-support

### Urgent neurological changes

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Sources: nccp · nccn-brain-support

### Steroids need a separate written plan

Steroids may be prescribed to reduce swelling around the tumour. They are not automatically needed for everyone. Ask for the exact dose, time, review date and any taper plan. Do not stop or change them suddenly without advice. Report sleep or mood changes, increased thirst, infection symptoms or muscle weakness and ask which additional checks or protection you need.

Sources: nccn-brain-support · nhs-dexamethasone

### Seizure medicines and rescue instructions

If prescribed, take seizure medicines on the agreed schedule. Ask what to do after a missed or vomited dose and whom to contact for breakthrough symptoms. Some medicines can affect mood, alertness or interact with treatment. Do not start, stop or switch them yourself. A rescue medicine needs an individual plan and training for whoever may give it.

Sources: nccn-brain-support

### Safety at home and outside

Ask your clinical team about driving, work, bathing, swimming, heights and machinery in light of your symptoms and any seizures. Local medical and licensing rules need individual advice; this guide does not give a universal driving clearance date. Occupational therapy can help adapt daily routines and the home.

Sources: nccn-brain-support

TREATMENT REFERENCE · 07

# Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

### Understand the sequence

Surgery or biopsy, radiotherapy, chemotherapy and follow-up have different jobs. Ask your team to draw the intended sequence and identify where you are now. Concurrent treatment means treatments overlap; adjuvant treatment follows the main treatment. A change of phase may change the dose, blood tests and support medicines.

Sources: nccn-brain-support

### Agree what treatment is trying to achieve

Ask whether the aim is to control the tumour, delay growth, relieve symptoms, preserve function or a combination of these. Discuss likely benefits, burdens, alternatives and what would prompt a pause or stop. The answer depends on the exact diagnosis, previous treatment and your priorities.

Sources: nccn-brain-support

### Put evidence in context

Clinical trials describe groups of people and may use older pathology names or different schedules. They cannot predict one person’s response or lifespan. Ask how closely the evidence matches your tumour and treatment. No benefit number from one schedule should automatically be applied to another.

Sources: nccn-brain-support

### Discuss options available to you

Ask about appropriate clinical trials, specialist opinions, symptom support and other treatment options. A medicine or device described in an international guide is not automatically licensed, funded or suitable in Ireland. The treating multidisciplinary team explains what is available and relevant.

Sources: nccn-brain-support

### Make room for your priorities

Bring up fatigue, thinking, independence, family time, work, travel and side effects you most want to avoid. Ask for information at a pace and in a format that suits you. You may ask for a trusted person to join the discussion, written notes and time to reconsider decisions.

Sources: nccn-brain-support

TREATMENT REFERENCE · 08

# Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

### Thinking, communication and daily tasks

Tell the team about word-finding, attention, memory, reading, vision or planning difficulties. Rehabilitation may include physiotherapy, occupational therapy, speech and language therapy, or neuropsychological support. Written routines, rest breaks and help with medicine organisation can make day-to-day care more manageable.

Sources: nccn-brain-support

### Food, nausea, bowels and energy

Report poor appetite, weight change, vomiting or constipation early. Ask for dietitian help if eating is difficult. Use the prescribed anti-sickness and bowel plan; do not wait until symptoms are severe. Choose activity that is safe for your balance and strength and balance it with rest. Ask about treatable causes of fatigue.

Sources: nccp · nccn-brain-support

### Emotional support and carers

Anxiety, low mood, changes in behaviour and uncertainty can affect both patients and carers. Tell the team what has changed and ask for appropriate psychological or social support. Carers may need their own information, practical help and breaks. Agree who the team may share information with.

Sources: nccn-brain-support

### Work, transport and finances

Ask the specialist nurse or social worker about getting to treatment, time off work, benefits, caring responsibilities and support at home. Plan who can help with medicines, symptom recording and appointments if your memory, vision or fatigue makes these difficult.

Sources: nccn-brain-support

### Supportive care and future wishes

Supportive and palliative care can help with symptoms and quality of life while active treatment continues. You can discuss future care preferences and who you want involved in decisions at a time that suits you. Ask the team for Irish advice and documentation rather than using legal or hospice rules from a US booklet.

Sources: nccn-brain-support

### Information that works on paper

Keep the printed schedule, current medicine list, tests, contact numbers and emergency instructions together. This handbook can be read in sequence without internet access. The wallet card and optional QR code help others find the regimen information; they do not replace your prescription or emergency plan.

Sources: nccn-brain-support

## CLV evidence: adult context and paediatric origins

The NCCP CLV regimen cites studies of cisplatin, lomustine and vincristine with radiotherapy in children with medulloblastoma.

The treatment strategy has a role in specialist multimodal care, but paediatric outcomes cannot be used as an adult survival estimate or applied to every embryonal tumour.

Sources: nccp · packer1994 · nci-medulloblastoma

PREPARE FOR THE CONVERSATION

## Questions worth asking

### What is my exact diagnosis and how does this regimen fit?

NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma. Ask for the pathology, relevant molecular findings and the treatment goal in plain language.

### Which medicines do I receive on which days?

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

### Which tests must happen before the next treatment?

Before starting: Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. During treatment: Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. Brain/spine imaging and any other response tests follow the individual specialist plan; no universal scan or tumour-marker timetable is set here.

**How do I handle a missed, vomited or held dose?**

Contact the oncology team or pharmacist for medicine-specific instructions. Do not double a dose or replace treatment that was held for toxicity.

**Who helps with steroids, seizures, hearing, memory or daily tasks?**

Ask for written medicine plans and a named contact. Rehabilitation, practical support and a trusted helper may support the daily plan.

**Which symptoms mean I should call now?**

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call promptly for fever, bleeding, new cough or breathlessness, persistent vomiting or reduced urine. Report hearing, nerve or bowel changes before the next infusion.

**When do we review whether treatment is helping?**

Agree the next clinical and imaging review, what would count as benefit, how side effects will be weighed and what would lead to continuing, changing or stopping.

> **Try explaining the plan back:** Can you show the day for each medicine, your next safety tests, the current support plan and whom to call for fever, new neurological symptoms or another urgent change?

SOURCE TRANSPARENCY

## Where this information comes from

Regimen schedule: NCCP 00806 v2. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.


- NCCP 00806 v2 · current Irish regimen
- NCI: medulloblastoma diagnosis, risk groups and treatment · patient education
- NCI: adult central nervous system tumours · diagnosis and grade education
- NHS: dexamethasone · use and stopping safely
- Packer 1994: childhood medulloblastoma study cited by NCCP · not an adult survival estimate
- HPRA: cisplatin Accord current product information · July 2026
- NCCN patient glioma guide 2026 · general brain-symptom, rehabilitation and carer concepts only; not CLV diagnosis or treatment evidence
- Lomustine medac current product information · January 2026

## Written source addresses

https://healthservice.hse.ie/documents/6870/806_V2_CLV_Therapy.pdf

https://www.cancer.gov/rare-brain-spine-tumor/tumors/medulloblastoma

https://assets.hpra.ie/products/Human/27566/Licence_PA2315-081-001_10072026160843.pdf

https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf

https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq

https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/

https://pubmed.ncbi.nlm.nih.gov/7931615/

https://www.nccn.org/patients/guidelines/content/PDF/brain-gliomas-patient.pdf